

Bayesian Basket Trial Design with False Discovery Rate Control

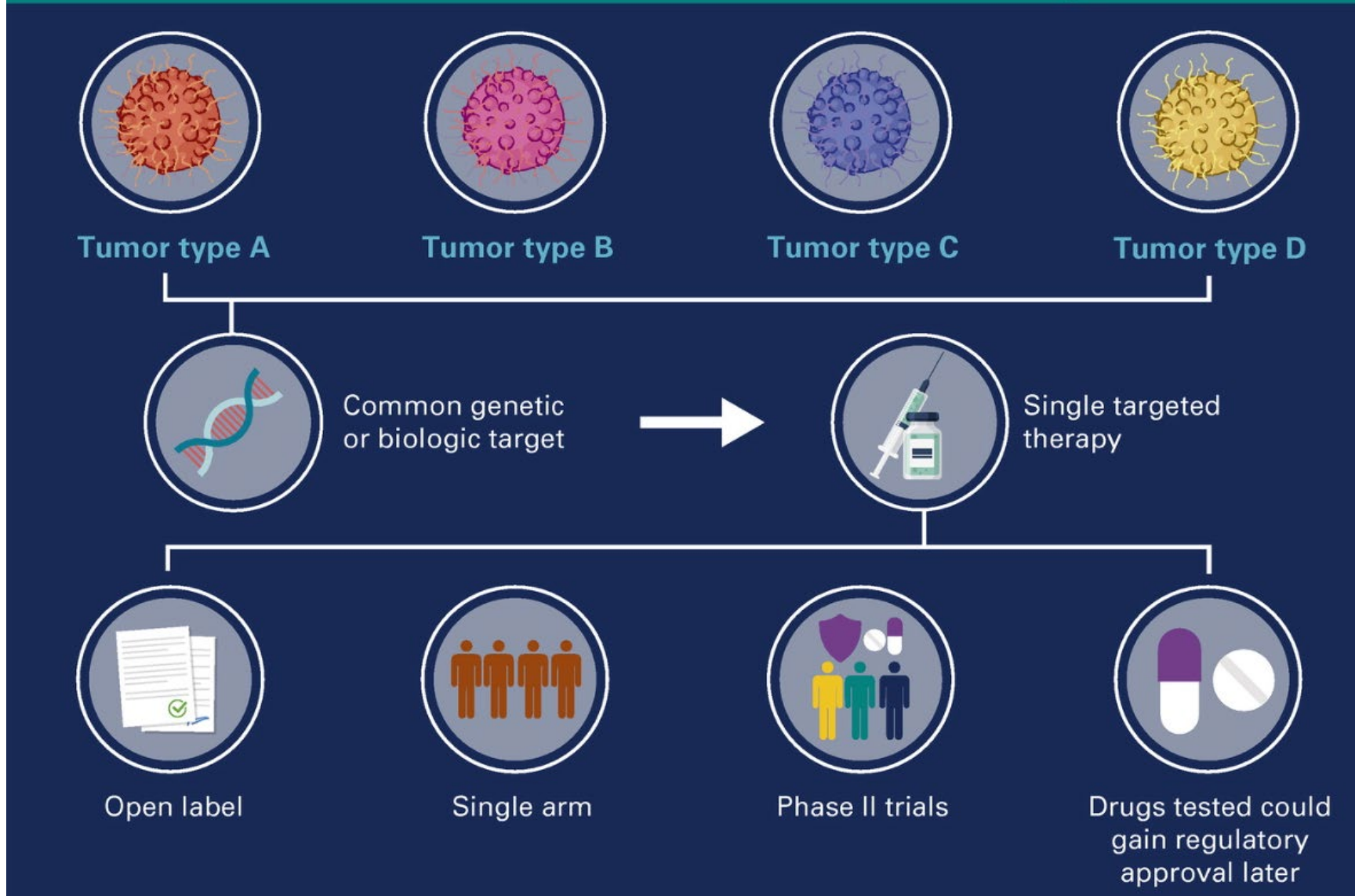
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1. **Umbrella trials:** study multiple targeted therapies in the context of a single disease
2. **Basket trials:** study a single targeted therapy in the context of multiple diseases or disease subtypes
3. **Platform trials:** study multiple targeted therapies in the context of a single disease in an adaptive, perpetual manner

Conventional Basket Trial Design



Hobbs, B. P., Pestana, R. C., Zabor, E. C., Kaizer, A. M., & Hong, D. S. (2022). Basket Trials: Review of Current Practice and Innovations for Future Trials. *Journal of Clinical Oncology*.

Advantages



★ Efficient enrollment of subjects



★ Helpful in the study of rare tumor types



★ Streamlined avenue for drug development and regulatory approval

Challenges



Traditional designs and statistical analyses assume exchangeable patients



Sensitive to enrollment trends because of the heterogeneous nature of subjects



Risk of type 1 statistical errors (false positive)

Basket-wise

Family-wise



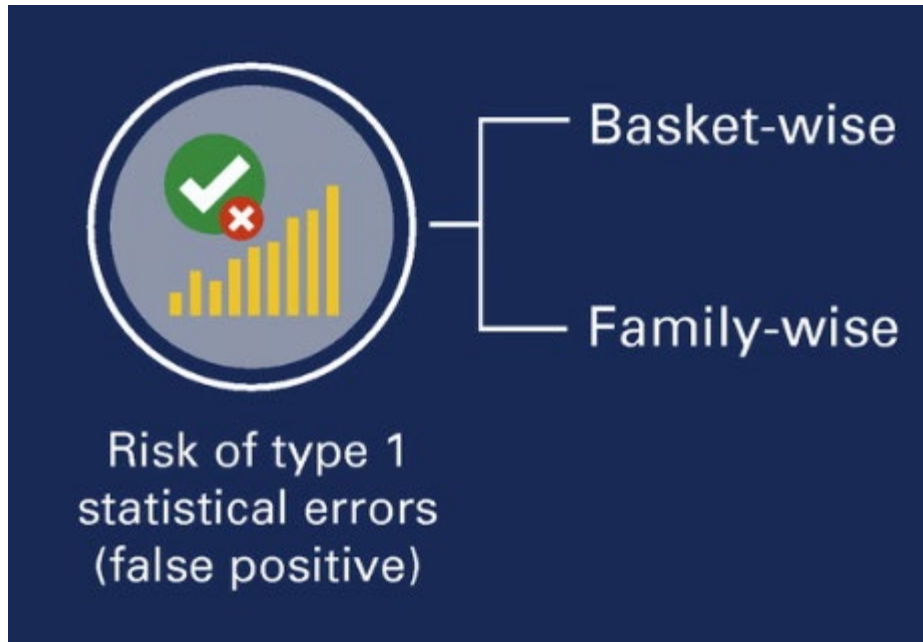
Traditional designs and statistical analyses assume exchangeable patients

Pooled analysis across all baskets may miss heterogeneity signals, and ignores subtype-specific differences in outcomes.



Sensitive to enrollment trends
because of the heterogenous
nature of subjects

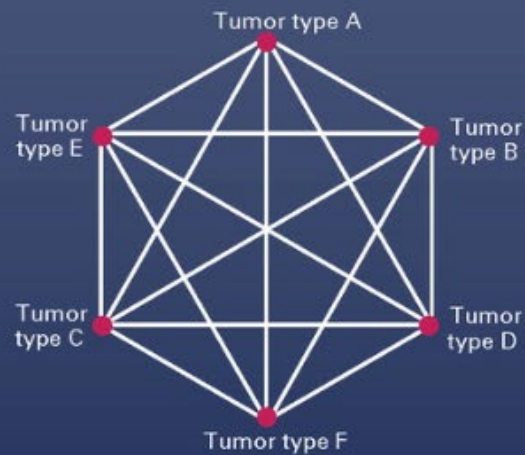
Enrollment trends may preclude evaluation of baskets with low enrollment or lead to erroneous conclusions for sparsely enrolled tumor types in the presence of heterogeneity.



Independent analysis of each basket **inflates the type I error rate.**

Potential Solutions

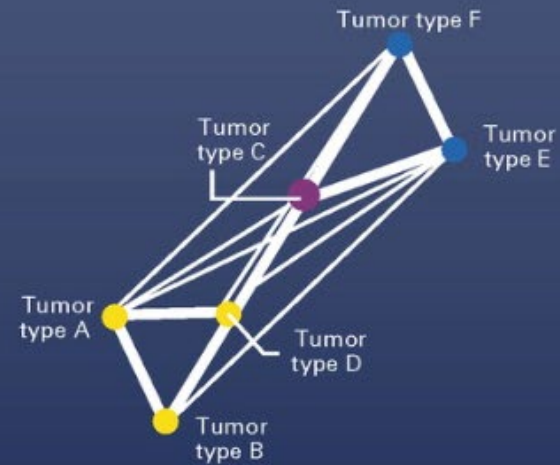
Prior distribution reflecting the tumor-agnostic effectiveness



Baseline assumptions ($n = 0$)

Posterior probability 0.204

Posterior distribution after observing data (heterogeneity)



Final data ($n = 84$)

Posterior probability 0.75 0.50 0.25

Bayesian methodology for ascertaining tumor heterogeneity

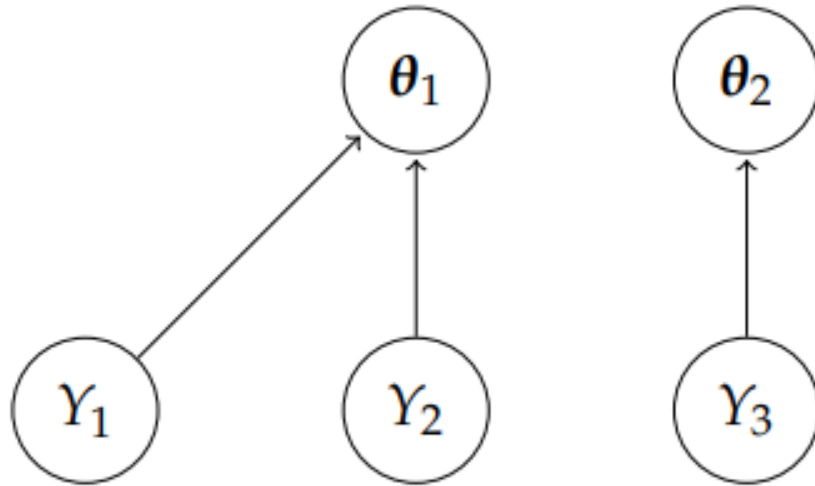
- Evidence for tumor-agnostic efficacy is measured

- Evidence is integrated in the presence of imbalanced enrollment

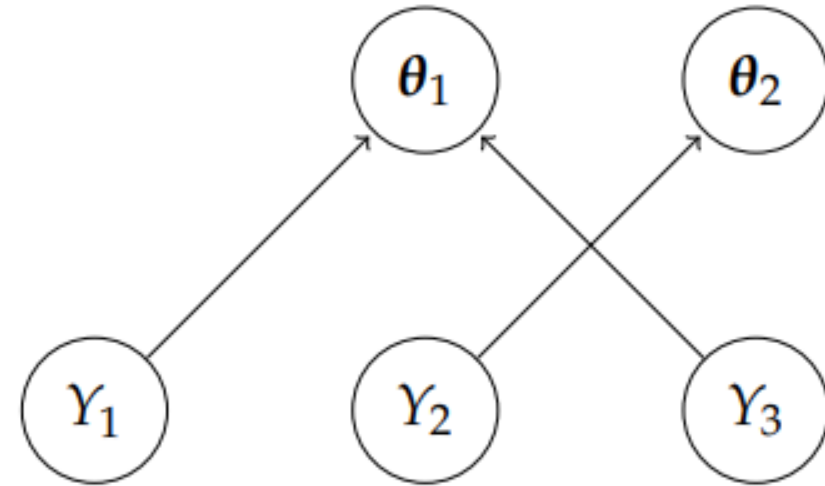
- Posterior decisions are robust to heterogeneity

- Weighted operating characteristics are used for design calibration

The multisource exchangeability model (MEM):



(a) Model where Y_1 and Y_2 are exchangeable.



(b) Model where Y_1 and Y_3 are exchangeable.

Kaizer, A. M., Koopmeiners, J. S., & Hobbs, B. P. (2018). Bayesian hierarchical modeling based on multisource exchangeability. *Biostatistics (Oxford, England)*, 19(2), 169–184.

Hobbs, B. P., & Landin, R. (2018). Bayesian basket trial design with exchangeability monitoring. *Statistics in medicine*, 37(25), 3557–3572.

basket

lifecycle **stable** build **passing** build **failing** codecov 78% R-CMD-check **failing**

Basket designs are prospective clinical trials that are devised with the hypothesis that the presence of selected molecular features determine a patient's subsequent response to a particular "targeted" treatment strategy. Basket trials are designed to enroll multiple clinical subpopulations to which it is assumed that the therapy in question offers beneficial efficacy in the presence of the targeted molecular profile. The treatment, however, may not offer acceptable efficacy to all subpopulations enrolled. Moreover, for rare disease settings, such as oncology wherein these trials have become popular, marginal measures of statistical evidence are difficult to interpret for sparsely enrolled subpopulations. Consequently, basket trials pose challenges to the traditional paradigm for trial design, which assumes inter-patient exchangeability. The R-package `basket` facilitates the analysis of basket trials by implementing multi-source exchangeability models. By evaluating all possible pairwise exchangeability relationships, this hierarchical modeling framework facilitates Bayesian posterior shrinkage among a collection of discrete and pre-specified subpopulations.

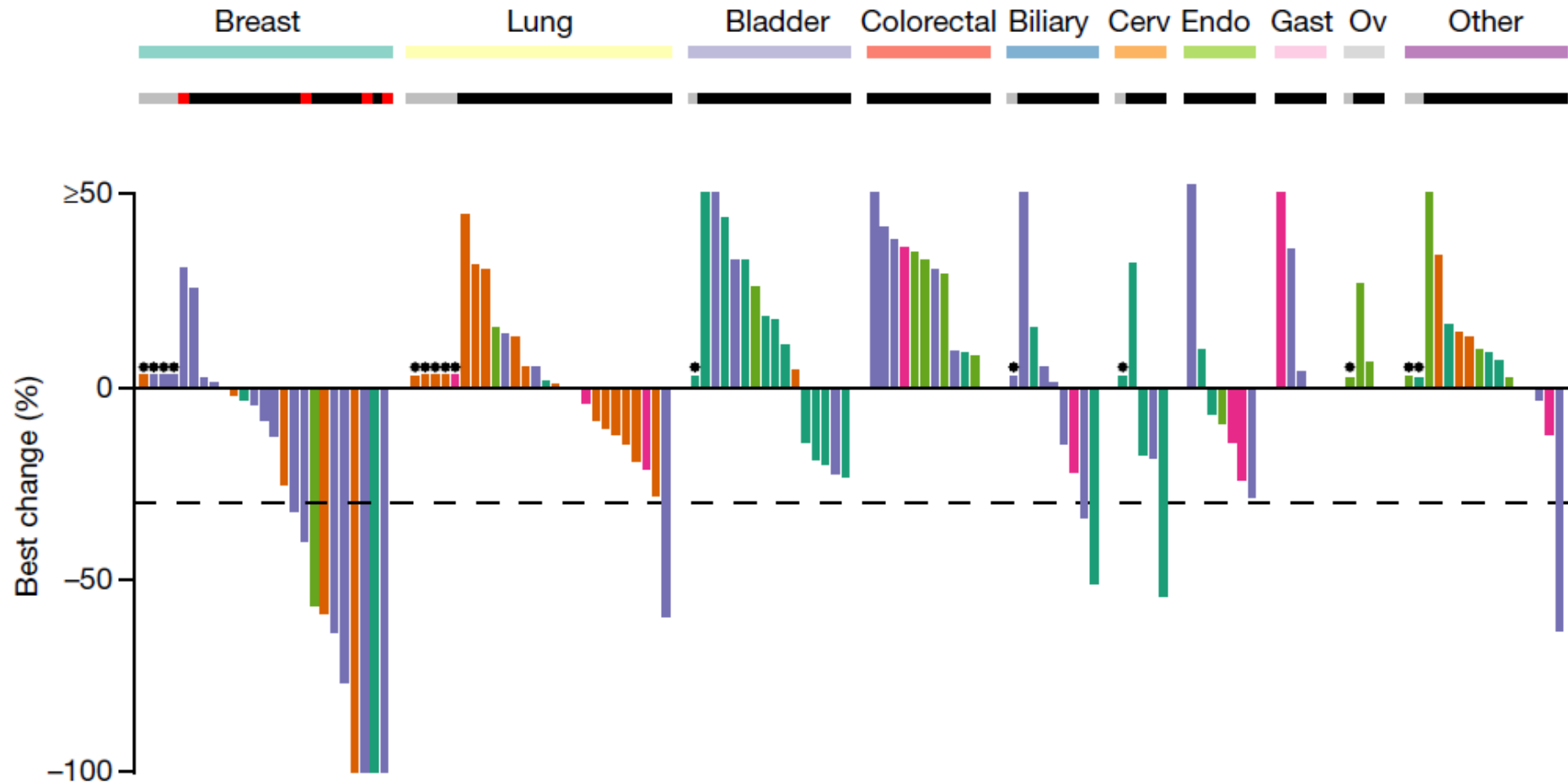
Installation

You can install the released version of `basket` from [CRAN](#) with:

```
install.packages("basket")
```

The **SUMMIT trial** tested neratinib in HER2- and HER3-mutant tumors in **10 subtypes**

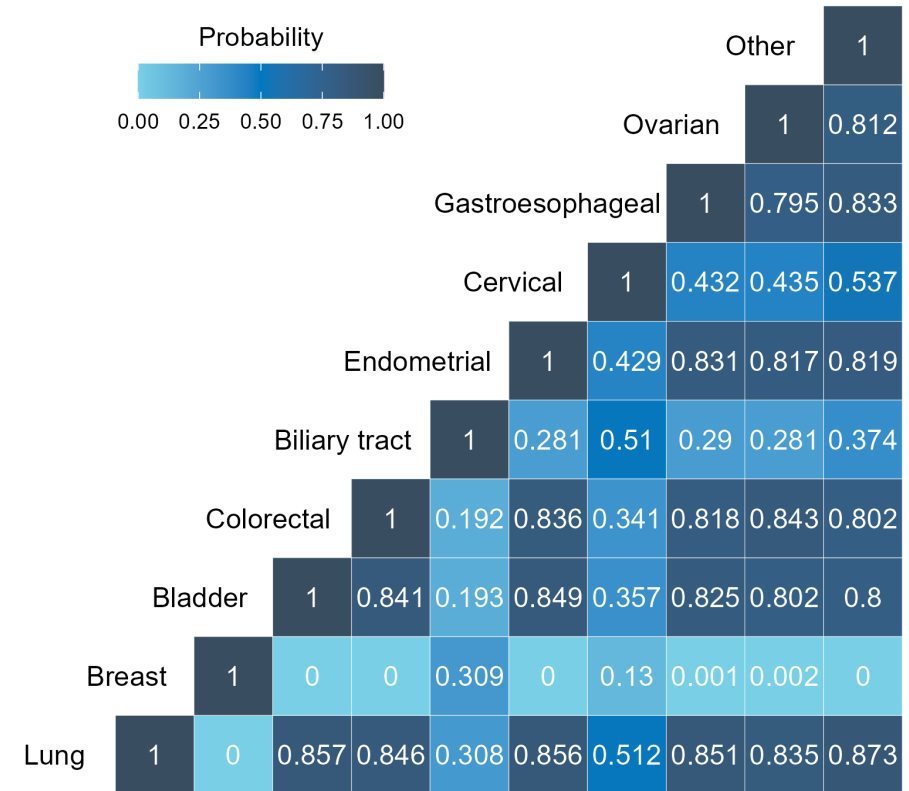
- ORR $\leq 10\%$ unacceptable, ORR $\geq 30\%$ acceptable
- **Independent** Simon's optimal two-stage designs
 - Enroll 7 patients
 - If at least 1 response, enroll additional 11
 - If 4 responses in 18 total, reject null

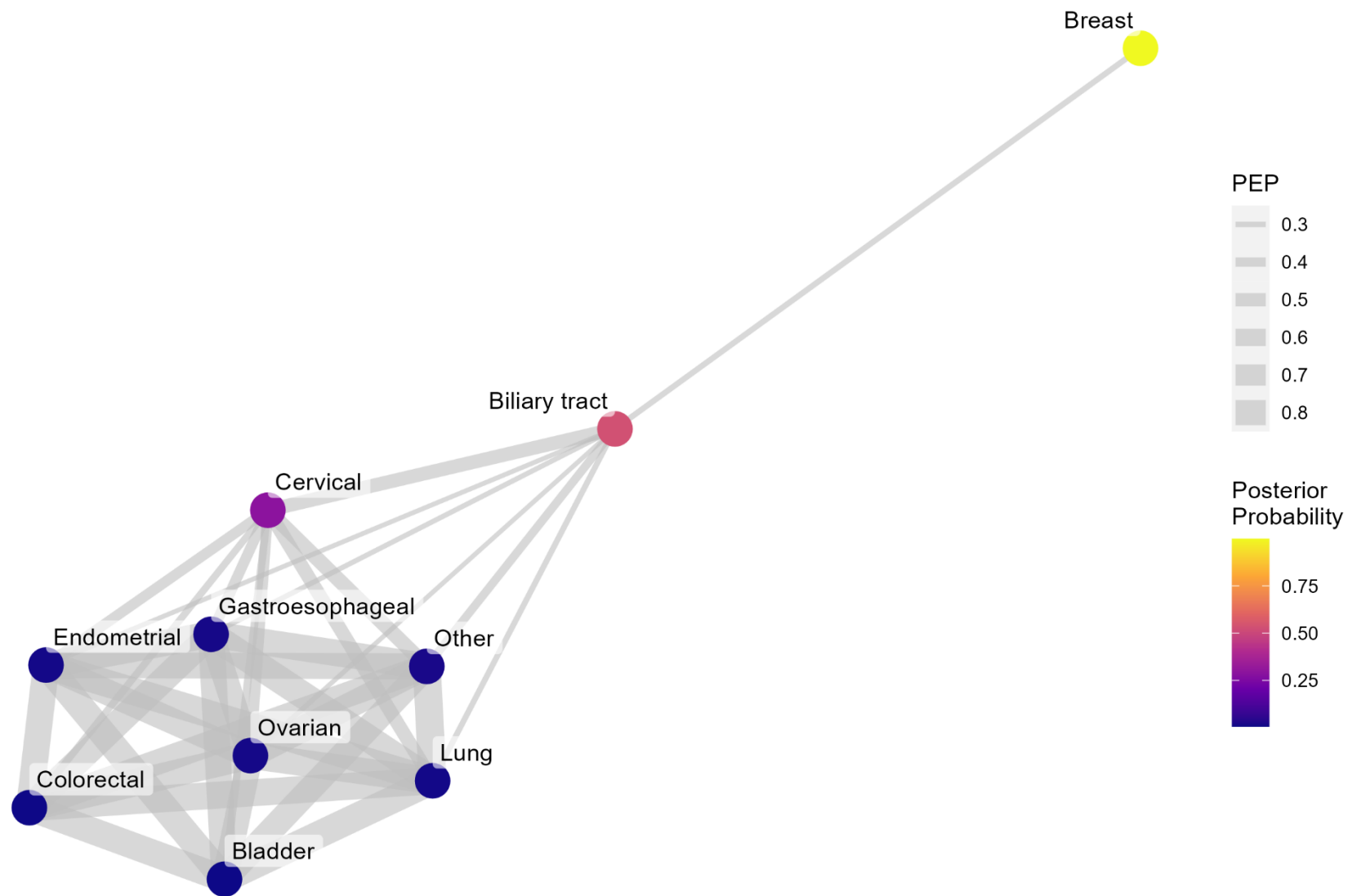


Maximum A Posteriori MEM

Other									1
Ovarian								1	1
Gastroesophageal							1	1	1
Cervical						1	0	1	0
Endometrial					1	0	1	1	1
Biliary tract				1	0	1	0	0	0
Colorectal			1	0	1	0	1	1	1
Bladder		1	1	0	1	0	1	1	1
Breast	1	0	0	1	0	0	0	0	0
Lung	1	0	1	1	0	1	0	1	1

Posterior Exchangeability Probability





Zabor, E. C., Kane, M. J., Roychoudhury, S., Nie, L., & Hobbs, B. P. (2022). Bayesian basket trial design with false-discovery rate control. *Clinical trials (London, England)*, 19(3), 297–306.

False Discovery Rate (FDR) control:

1. Obtain the posterior probability that each basket j exceeds the null response rate, $\Pr(\pi_j > \pi_0 | \mathbf{S})$
2. Order the posterior probabilities from largest to smallest, $\Pr(\pi_j > \pi_0 | \mathbf{S})_{(1)}, \dots, \Pr(\pi_j > \pi_0 | \mathbf{S})_{(J)}$
3. For a given threshold of posterior probability, ϕ , identify the largest k such that $\Pr(\pi_j > \pi_0 | \mathbf{S})_{(k)} > \frac{k}{J} \times \phi$
4. Declare all baskets $\Pr(\pi_j > \pi_0 | \mathbf{S})_{(i)}, i = 1, \dots, k$ to be significant at threshold ϕ

Basket	N	ESS	PP	Rank (k)	$\frac{k}{J} \times \phi$	Significant*
Breast	25	27.9	0.999	1	0.095	TRUE
Biliary tract	11	16.0	0.531	2	0.190	TRUE
Cervical	5	15.4	0.291	3	0.285	TRUE
Other	19	91.7	0.020	4	0.380	FALSE
Gastroesophageal	7	90.4	0.016	5	0.475	FALSE
Ovarian	5	90.8	0.015	6	0.570	FALSE
Endometrial	8	94.6	0.013	7	0.665	FALSE
Lung	26	97.5	0.012	8	0.760	FALSE
Colorectal	17	100.0	0.007	9	0.855	FALSE
Bladder	18	102.0	0.007	10	0.950	FALSE

*At posterior threshold $\phi = 0.95$

Table 1. True response probabilities used to compare trial operating characteristics between Bayesian design with MEM and independent frequentist design.

Scenario	Basket									
	Lung n=26	Breast n=25	Bladder n=18	CRC n=17	Biliary n=11	EC n=8	Cervical n=5	GE n=7	Ovarian n=5	Other n=19
Global Alt	0.30	0.30	0.30	0.30	0.30	0.30	0.30	0.30	0.30	0.30
Mixed Alt 1	0.10	0.30	0.10	0.10	0.10	0.10	0.10	0.10	0.10	0.10
Mixed Alt 2	0.10	0.30	0.10	0.10	0.30	0.10	0.10	0.10	0.10	0.10
Mixed Alt 3	0.10	0.30	0.10	0.10	0.30	0.10	0.30	0.10	0.10	0.10
Global Null	0.10	0.10	0.10	0.10	0.10	0.10	0.10	0.10	0.10	0.10

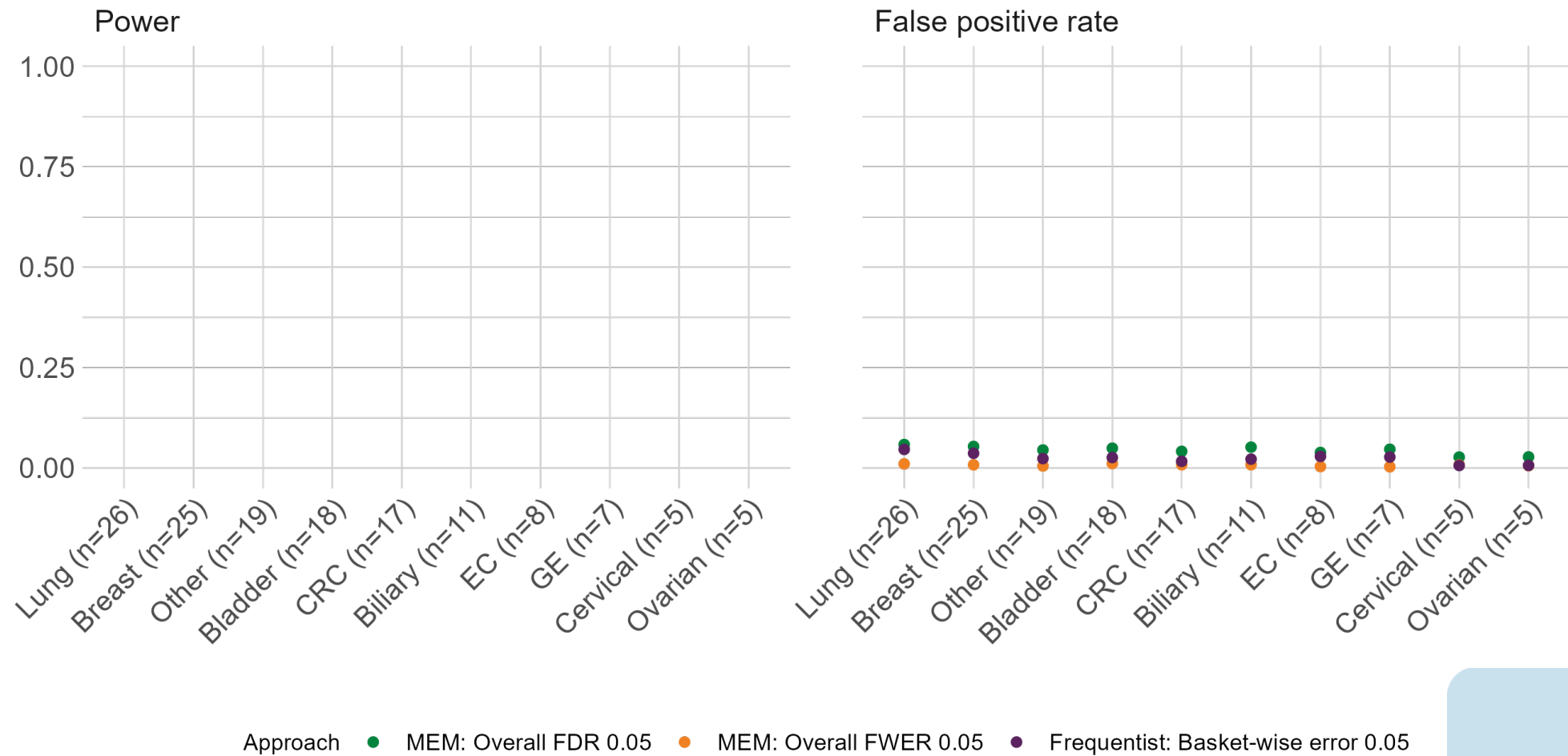
Alt = Alternative; CRC=Colorectal; EC=Endometrial; GE=Gastroesophageal

Table 2. Power, false positive rate (FPR) and family-wise error rate (FWER) for each scenario using the MEM approach.

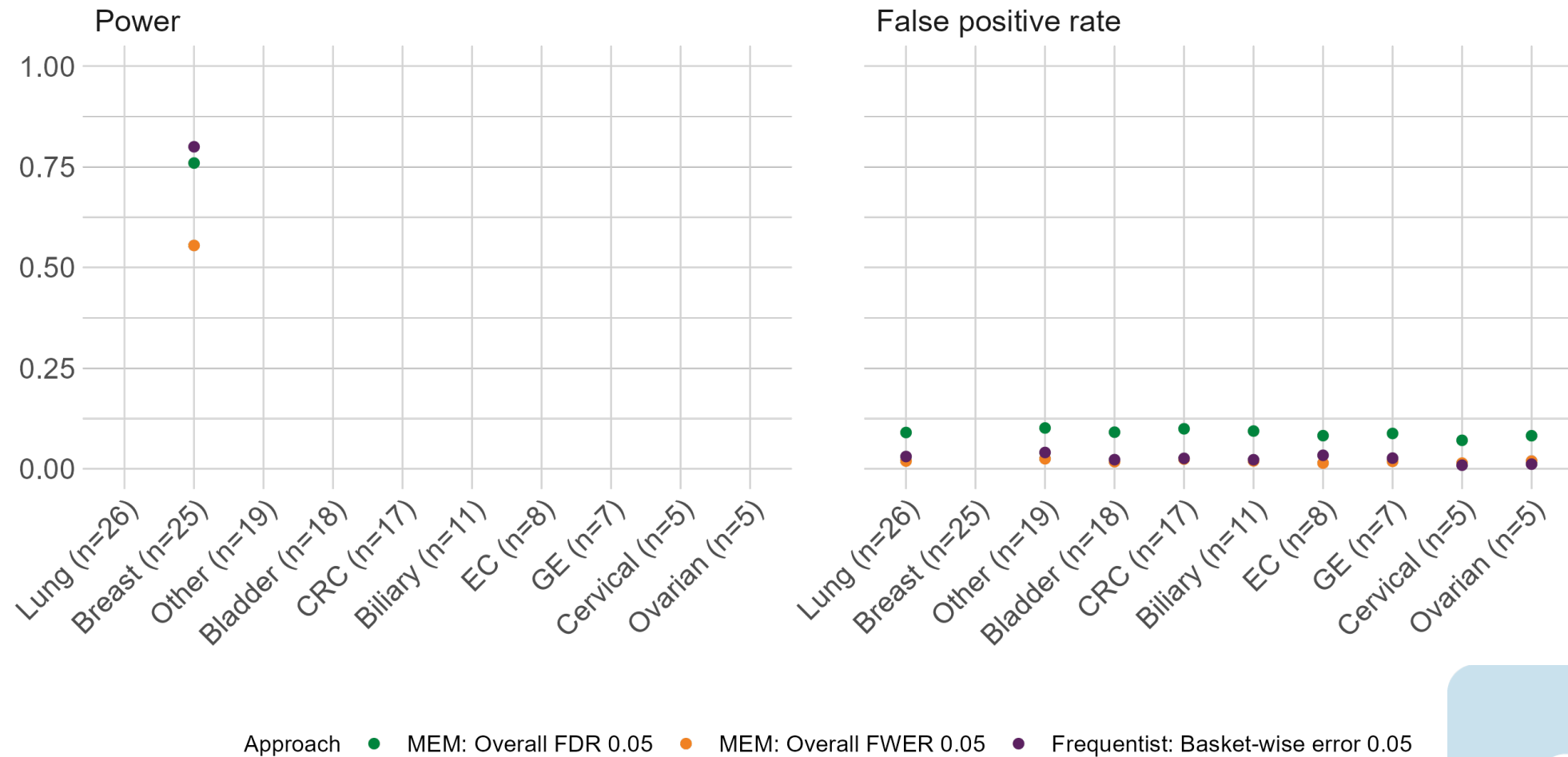
Setting	FDR = 0.1			FDR = 0.05			FWER = 0.05	
	Power	FPR	FWER	Power	FPR	FWER	Power	FPR
Global Null	–	0.10	0.32	–	0.04	0.17	–	0.01
Mixed Alternative 1	0.86	0.18	0.52	0.76	0.09	0.31	0.56	0.02
Mixed Alternative 2	0.81	0.24	0.58	0.69	0.12	0.38	0.48	0.03
Mixed Alternative 3	0.75	0.27	0.58	0.62	0.15	0.41	0.41	0.04
Global Alternative	0.96	–	–	0.94	–	–	0.90	–

*Posterior threshold 0.867 controls the FDR at 0.1; posterior threshold 0.946 controls the FDR at 0.05

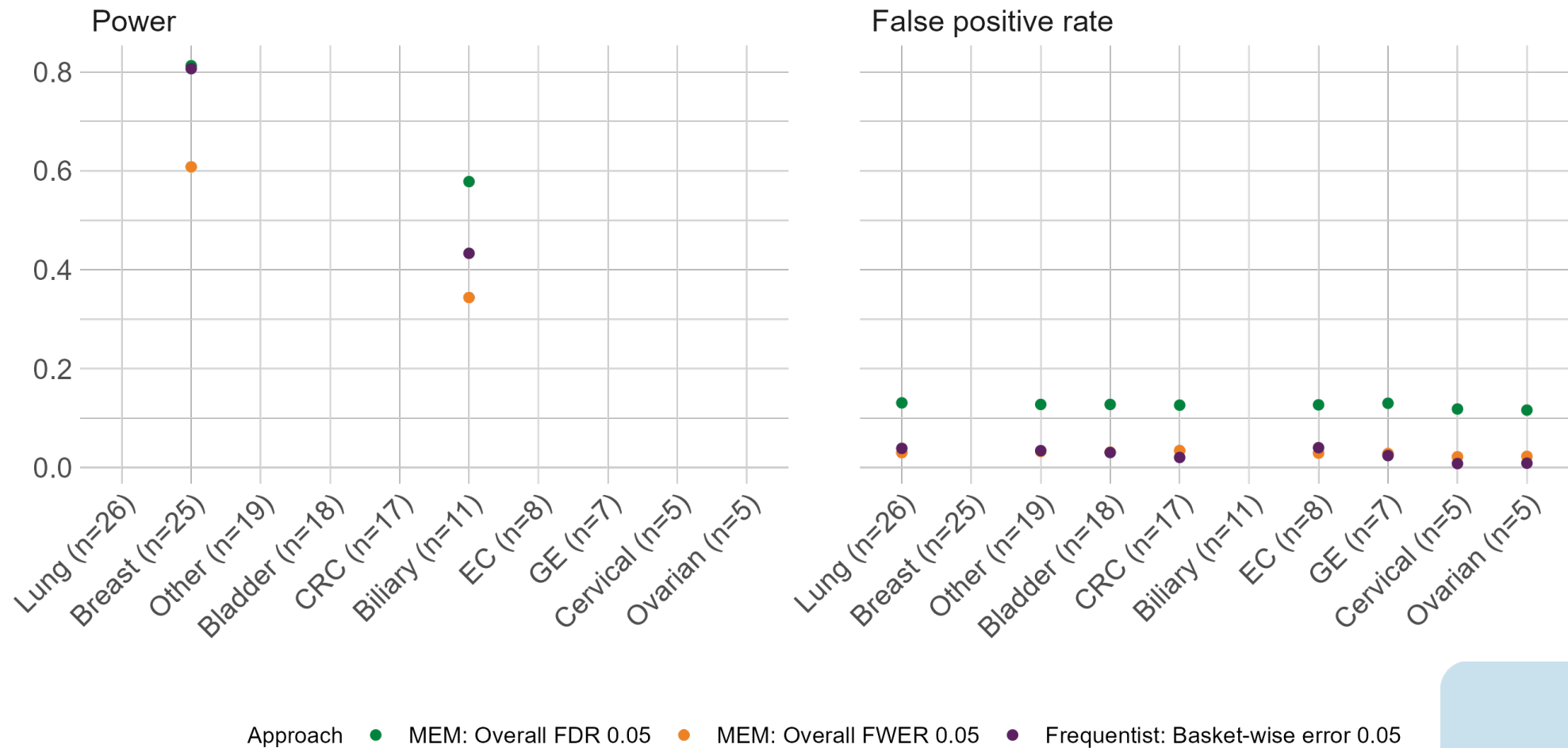
Global Null



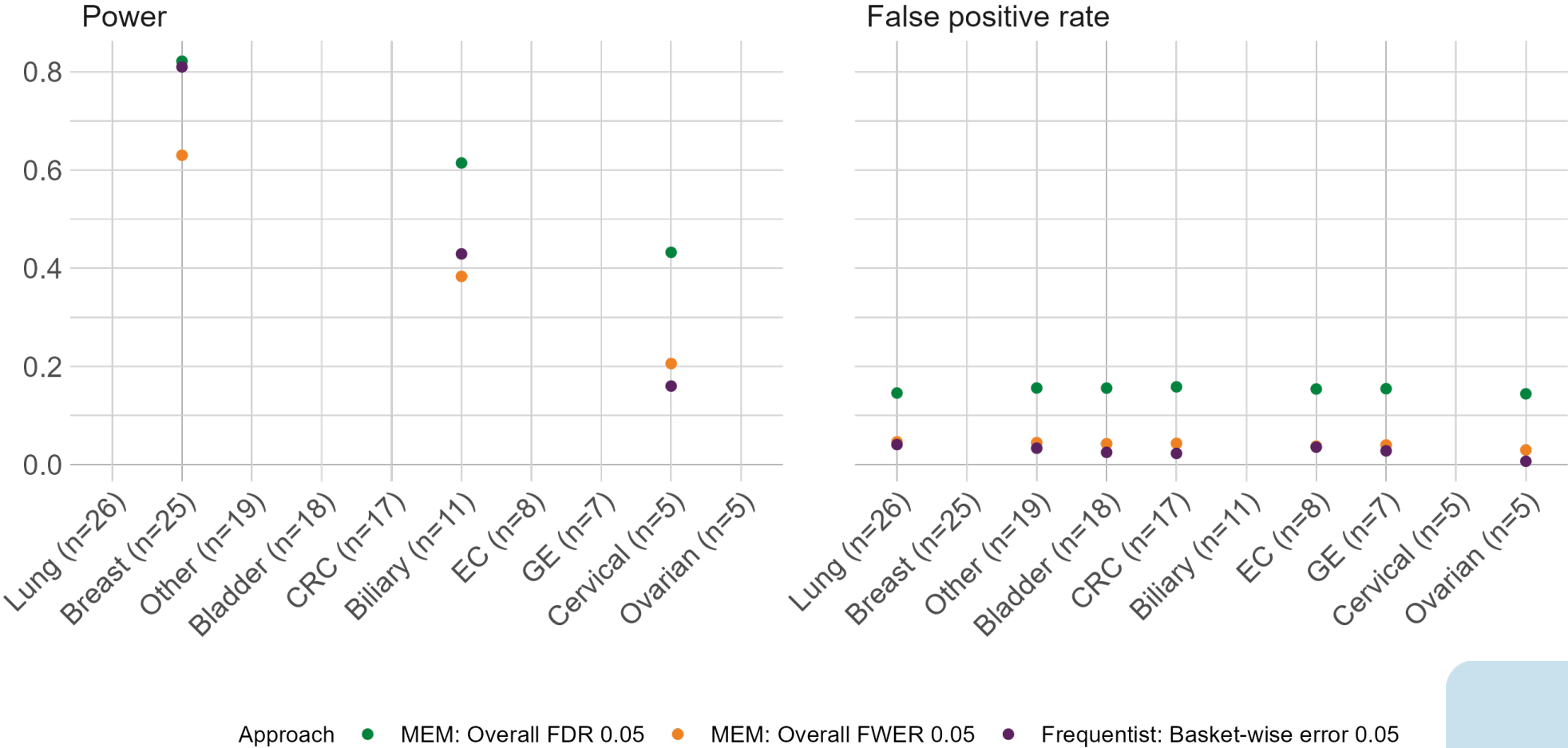
Mixed Alternative 1



Mixed Alternative 2

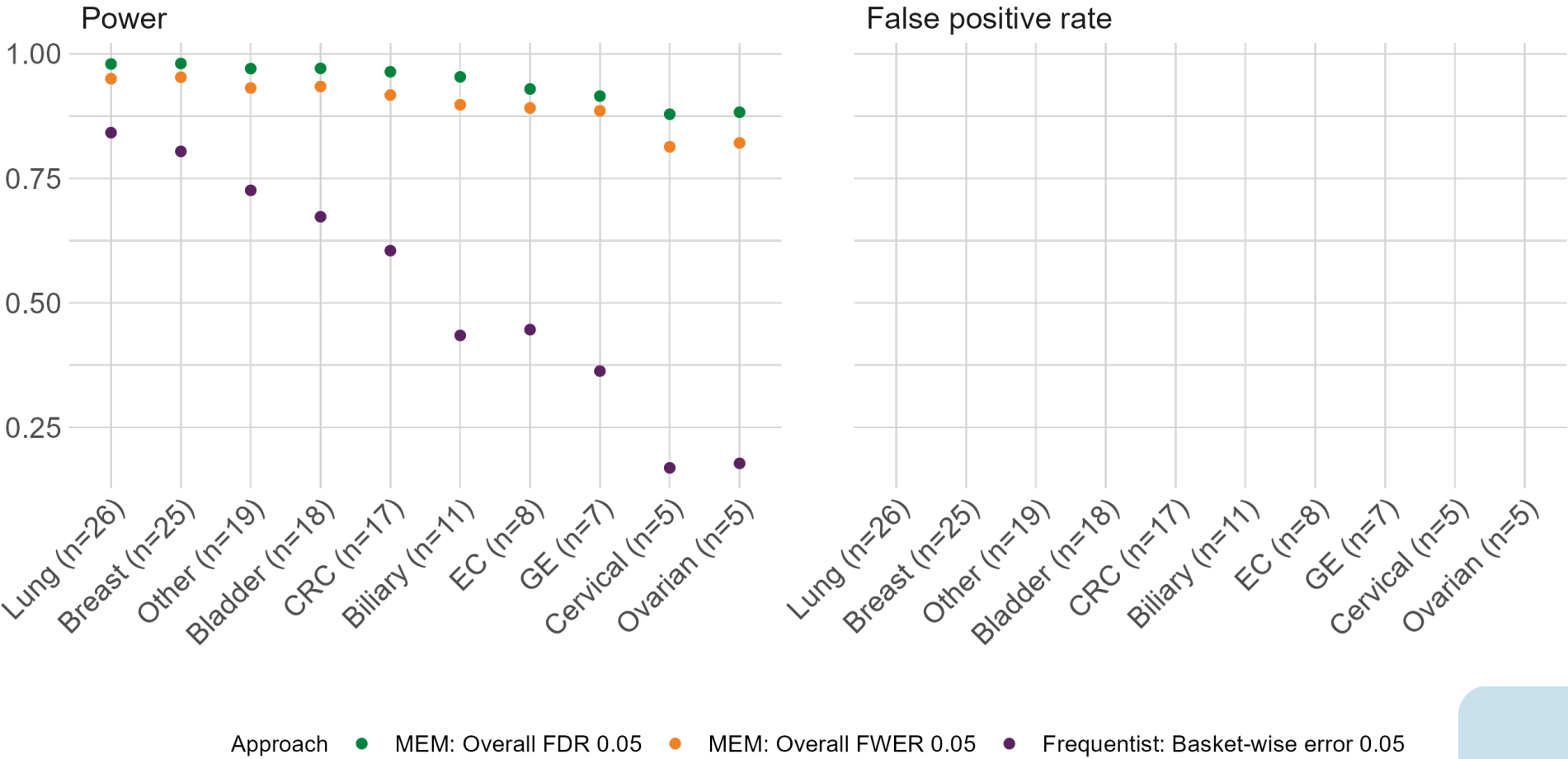


Mixed Alternative 3



Approach ● MEM: Overall FDR 0.05 ● MEM: Overall FWER 0.05 ● Frequentist: Basket-wise error 0.05

Global Alternative



Summary

- MEM information-sharing **compensates for small basket sample sizes** and improves power
- MEM **directly examines heterogeneity** across baskets through posterior exchangeability probabilities
- FDR control **maintains desired higher power** at the cost of higher false positive rate
- Easy implementation with the **basket package in R**



Questions?



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[@zabormetrics](https://twitter.com/zabormetrics)



[@zabore](https://github.com/zabore)



References

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5. Kane, M. J., Chen, N., Kaizer, A. M., Jiang, X., Xia, H. A., & Hobbs, B. P. (2020). Analyzing Basket Trials under Multisource Exchangeability Assumptions. *The R Journal*, 12(2).
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